

Schizophrenia

TC: TC-0033 (TC)

[Link to Codes](#)**MCG Health**Transitions of Care
28th Edition

- Patient Interview
 - Self-Care
 - Activity
 - Nutrition
 - Medicine
 - Complications
 - Follow-Up Care
- Case Management Evaluation
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Patient Interview

Self-Care

Q1. Patient: What are you doing to keep another schizophrenia episode from happening? (Check all that apply.)

- Adhere to self-care plan
- Ask for help when feelings are overwhelming
- Counseling or therapy attendance
- Exercise program
- Follow a daily care plan
- Follow up with doctors
- Get all follow-up laboratory tests when due
- Keep a diary about feelings, activities, eating times, and medicines taken
- Keep all doctor appointments
- Know when to contact the doctor or therapist
- Lose weight [Q2]
- Manage situation or event triggers
- Manage stress
- Meal planning (diet) changes
- Seek emergency care for signs of crisis plan not working
- Set realistic expectations for symptom management and well-being
- Stop using alcohol, tobacco, or drugs
- Take medicine
- Vaccinations
- Other, specify: _____ [P1]
- Nothing [P1]
- Unknown [P1] [P2]
- Not all care measures understood [P2]

Q2. Patient: What are you doing to lose weight? (Check all that apply.)

- Avoid sweets
- Be more active
- Eat fewer calories or less fat
- Meal planning (diet) changes
- Skip meals [P1] [P2] Nutrition Assessment, Adult [TC](#)
- Take diet supplements [P1] [P2] Nutrition Assessment, Adult [TC](#)
- Other, specify: _____ [P1] [P2] Nutrition Assessment, Adult [TC](#)

- Nothing [P1] [P2] Nutrition Assessment, Adult [TC](#)
- Unknown [P1] [P2] Nutrition Assessment, Adult [TC](#)
- Not applicable

Q3. Patient: Do you keep a diary of your mood and feelings?

- Yes
- No [P2]
- Unknown [P2]

Q4. Patient: Do you use tobacco, alcohol, or drugs? (Check all that apply.)

- Yes, tobacco use [P2] Tobacco Use [TC](#)
- Yes, alcohol use [P2] [P3] Alcohol Use [TC](#)
- Yes, drug use [P2] [P3] Substance Use [TC](#)
- No
- Unknown [P1] Tobacco Use [TC](#) Alcohol Use [TC](#) Substance Use [TC](#)

Q5. Patient: Do you use any equipment or supplies to care for your condition?

- Yes, specify: _____ Equipment Assessment [TC](#)
- No
- Unknown

Q6. Patient: Are you having any problems with your self-care? (Check all that apply.)

- Affording medicine or supplies [P4] [P5] [P6] Financial Status and Benefits [TC](#)
- Behavior indicating cognitive impairment [P4] [P6] Cognitive Impairment [TC](#) Behavioral Health and Cognition Evaluation with Caregiver [TC](#)
- Blood pressure care [P4] [P6] Blood Pressure Monitoring [TC](#)
- Blood sugar care [P4] [P6] Blood Glucose Monitoring [TC](#)
- Blood thinner care [P4] [P6] Anticoagulation Management [TC](#)
- Bowel program [P4] [P6] Bowel Management [TC](#)
- Caregiver help needed [P4] [P6] [P7] Caregiver Resources Evaluation [TC](#)
- Community resource information needed [P4] [P6] Community Resources [TC](#)
- Exercising and staying active [P4] [P6] Exercise [TC](#)
- Fatigue or weakness [P4] [P6] Fatigue or Weakness [TC](#)
- Pain [P4] [P6] Pain Assessment [TC](#)
- Planning for long-term care needs [P4] [P6] Advance Care Planning [TC](#) Palliative or End-of-Life Evaluation [TC](#)
- Safety in the home [P4] [P6] Home Safety Evaluation [TC](#)
- Skin care management [P4] [P6] Pressure Injury Monitoring and Management [TC](#)
- Social support [P4] [P6] Social Support [TC](#)
- Taking medicine as prescribed [P4] [P6] [P8] Medication Adherence [TC](#)
- Transportation for care [P4] [P6] [P9] Transportation [TC](#)
- Trouble gaining weight [P4] [P6] Nutritional Status, Adult [TC](#) Nutritional Management to Gain Weight [TC](#)
- Trouble losing weight [P4] [P6] Nutritional Status, Adult [TC](#) Nutritional Management to Lose Weight [TC](#)
- Trouble making lifestyle changes [P4] [P6] Change Readiness [TC](#)
- Trouble understanding how to do self-care [P4] [P6] Health Literacy [TC](#)
- Other, specify: _____ [P4] [P6]
- No
- Unknown [P4] [P6]

Activity

Q1. Patient: What do you do to stay active? (Check all that apply.)

- Active range of motion exercises [Q2] [Q3]
- Aerobic exercise (eg, jogging, swimming, dancing) [Q2] [Q3]
- Low-impact exercises [Q2] [Q3]

- Strength training exercises [Q2] [Q3]
- Other, specify: _____ [Q2] [Q3]
- Nothing [P1] [P2]
- Unknown [Q2] [Q3] [P1] [P2]

Q2. Patient: How often do you exercise and for how long? (Check all that apply.)

- At least 3 times a week
- 1 or 2 times a week [P1] [P2]
- Inconsistently [P1] [P2]
- More than 30 minutes each session
- 15 to 30 minutes each session
- Less than 15 minutes each session [P1] [P2]
- Other, specify: _____ [P1] [P2]
- Unknown [P1] [P2]
- Not applicable

Q3. Patient: Do you have any problems when you exercise? (Check all that apply.)

- Chest pain [P1]
- Dizziness [P1]
- Trouble breathing [P1]
- Weakness [P1]
- Other, specify: _____ [P1]
- No
- Unknown [P2]
- Not applicable

Nutrition

Q1. Patient: What meal plan (diet) do you follow? (Check all that apply.)

- DASH Diet Knowledge Insufficient - DASH Diet [TC Nutrition Assessment, Adult](#) [TC](#)
- Heart healthy Diet Knowledge Insufficient - Heart Healthy [TC Nutrition Assessment, Adult](#) [TC](#)
- Kidney diet Diet Knowledge Insufficient - Renal Dysfunction [TC Nutrition Assessment, Adult](#) [TC](#)
- Low carb or low glycemic index Diet Knowledge Insufficient - Low Glycemic Index [TC Nutrition Assessment, Adult](#) [TC](#)
- Other, specify: _____ Nutrition Assessment, Adult [TC](#)
- None [P1] [P2] Nutrition Assessment, Adult [TC](#)
- Unknown [P1] [P2] Nutrition Assessment, Adult [TC](#)

Medicine

Q1. Patient: What medicine do you take for your schizophrenia? (Check all that apply.)

- Atypical antipsychotics (eg, aripiprazole, clozapine, risperidone, ziprasidone) [Q2] [P10] [P11]
- Injectable antipsychotics (eg, fluphenazine decanoate, haloperidol decanoate, risperidone) [Q2] [P10] [P12]
- Typical antipsychotics (eg, chlorpromazine, haloperidol, thioridazine) [Q2] [P10] [P12]
- Other, specify: _____ [Q2] [P10]
- None [P13]
- Unknown [P13]

Q2. Patient: Are you having side effects from your medicine? (Check all that apply.)

- Blurred vision [P8] [P10]
- Constipation [P8] [P10]
- Dizziness [P8] [P10]
- Dry mouth [P8] [P10]
- Fast heart rate [P8] [P10]
- Inability to urinate [P8] [P10]
- Involuntary repetitive movements (eg, grimacing, tongue movements, lip smacking, pursed lips, or excessive eye blinking) [P8] [P10]
- Light-headed when getting up from bed or chair [P8] [P10]
- Nausea or vomiting [P8] [P10]
- Skin rash [P8] [P10]
- Stiffness or muscle rigidity [P8] [P10]

- Tiredness [P8] [P10]
- Tremors [P8] [P10]
- Weight gain [P8] [P10]
- Other, specify: _____ [P8] [P10]
- None
- Unknown [P8] [P10]
- Not applicable

Q3. Patient: Are you taking any new medicines prescribed by the doctor?

- Yes, specify: _____ [P14]
- No, I do not have any new medicine.
- No, I haven't started taking the new medicine. Medication Adherence  TC
- Unknown Medication Adherence  TC

Q4. Patient: Are you taking any new medicines that you got without a prescription (eg, over-the-counter medicines, vitamins, minerals, or herbal or dietary supplements)?

- Yes, specify: _____ [P14]
- No
- Unknown

Q5. Patient: Are there any medicines that you are no longer taking?

- Yes, specify: _____ [P14]
- No
- Unknown

Q6. Patient: Are there any medicines that you are taking more or less of?

- Yes, specify: _____ [P14]
- No
- Unknown

Q7. Patient: Do you have a list of your current medicine information?

- Yes
- No [P14]
- Unknown

Q8. Patient: Do you have any medicines that have 1 refill or less?

- Yes [P8] [P15]
- No
- Unknown [P8] [P15]
- Not applicable

Q9. Patient: Do you ever miss taking your medicine?

- Yes [P8] Medication Adherence  TC
- No
- Unknown [P8] Medication Adherence  TC
- Not applicable

Complications

Q1. Patient: What are the signs of schizophrenia? (Check all that apply.)

- Confused thinking
- Delusions, like believing that others are controlling your mind or that someone is out to hurt you
- Difficulty having fun
- Difficulty showing emotion
- Difficulty socializing with others
- Feeling suspicious about others
- Feeling very irritable
- Hallucinations, like seeing things that are not there or hearing voices
- Talking that does not make sense
- Trouble completing activities
- Trouble concentrating or paying attention
- Trouble sleeping
- Other, specify: _____ [P2]

- None [P2]
- Unknown [P2]
- Not all symptoms understood [P2]

Q2. Patient: Are you currently having symptoms of schizophrenia?

- Yes [P1]
- No
- Unknown [P1] [P2]

Q3. Patient: What do you do if you have signs of schizophrenia? (Check all that apply.)

- Avoid things that make me feel worse
- Call behavioral health counselor or therapist
- Call support system for help
- Exercise or do activity
- Get medical care
- Manage stress
- Take medicine, specify: _____
- Other, specify: _____
- Nothing [P1] [P2]
- Unknown [P1] [P2]
- Not all care measures understood [P2]

Q4. Patient: Has your mood changed over the past 2 weeks?(1)(2)  PHQ-2 Calculator **QM**

- Yes [Q5]
- No
- Unknown

Q5. Patient: What changes in your mood have you had? (Check all that apply.) **QM**

- Change in usual activity (eg, working excessively, no longer talking to friends, or going on shopping sprees) Depression Screening, Adult  TC
- Down, hopeless, or depressed Depression Screening, Adult  TC
- Forgetting a lot or getting angry or frustrated Depression Screening, Adult  TC
- Little interest or pleasure in doing things Depression Screening, Adult  TC
- Quick mood changes Depression Screening, Adult  TC
- Other, specify: _____ Depression Screening, Adult  TC
- Not applicable

Follow-Up Care

Q1. Patient: Are you supposed to have surgery or a procedure? If yes, when is it scheduled? (Include scheduled date.)

- Electroconvulsive therapy (ECT), date scheduled: _____ Preoperative Care Coordination - Assessment and Planning  TC
- Other, specify: _____
- None
- Unknown [P2]

Q2. Patient: Has your doctor ordered any follow-up care? (Check all that apply.)

- Dietitian Outpatient Services - Assessment and Planning  TC
- Home care [P2] Outpatient Services - Assessment and Planning  TC
- Laboratory tests, specify: _____ [Q3]
- Community education program referral, specify: _____ Appointment Scheduling  TC
- Behavioral health provider referral, specify: _____ Appointment Scheduling  TC
- Psychiatrist referral, specify: _____ Appointment Scheduling  TC
- Social worker referral, specify: _____ Appointment Scheduling  TC
- Specialist referral, specify: _____ Appointment Scheduling  TC
- Rehabilitation [P2] Outpatient Services - Assessment and Planning  TC
- Other, specify: _____ [P2] Outpatient Services - Assessment and Planning  TC
- None [P1]

- Unknown [P2]

Q3. Patient: When is your laboratory work due?

- This week
- 1 to 2 weeks
- 3 to 4 weeks
- 1 to 3 months [P2]
- 4 to 11 months [P16]
- 1 year or more [P16]
- Other, specify: _____ [P2] [P16]
- Unknown [P1] [P16]

Case Management Evaluation

Q1. Case Manager: Does the patient have a written self-care plan?

- Yes
- No [P1] [P6]
- Unknown [P1] [P6]

Q2. Case Manager: Is the patient at risk for further complications? (Check all that apply.)

- Adhering to self-care plan [P1] [P4] Change Readiness [TC](#)
- Affording medication or supplies [P1] [P4] [P5] Financial Status and Benefits [TC](#)
- Behavior indicating cognitive impairment [P1] [P4] Behavioral Health and Cognition Evaluation with Caregiver [TC](#)
- Caregiver help needed [P1] [P4] [P7] [P17] Caregiver Resources Evaluation [TC](#)
- Community resource information needed [P1] [P4] Community Resources [TC](#)
- Exercising and staying active [P1] [P4] Exercise [TC](#)
- Health literacy [P1] [P4] Health Literacy [TC](#) Home Care Coordination - Assessment and Planning [TC](#)
- Medication nonadherence [P1] [P4] Medication Adherence [TC](#) Home Care Coordination - Assessment and Planning [TC](#)
- Pain [P1] [P4] Pain Assessment [TC](#)
- Rehospitalization [P1] [P4] Hospitalization Risk Evaluation [TC](#)
- Safety in the home [P1] [P4] Home Safety Evaluation [TC](#) Home Care Coordination - Assessment and Planning [TC](#)
- Self-care education or training needed [P1] [P4] [P6] Home Care Coordination - Assessment and Planning [TC](#)
- Social support [P1] [P4] Social Support [TC](#)
- Transportation for care [P1] [P4] [P9] Transportation [TC](#)
- Trouble making lifestyle changes [P1] [P4] Change Readiness [TC](#)
- Other, specify: _____ [P1] [P4]
- None identified
- Unknown [P1] [P4]

Q3. Case Manager: Is continued care coordination needed?

- Yes
- No Transitions of Care Case Management Closure [TC](#)
- Unknown [P4]

Quality Measures

- Disease Management Quality Measures:
 - Information provided from this question may be requested by various accrediting organizations.

Plan of Care

Problems

P1. Schizophrenia complication risk present

- **Goal:** Schizophrenia complication risk evaluated and managed **NNN**
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up

- **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
- **ASSIST:** Appointment scheduling for evaluation of problems, comorbidities, and self-care plan[A]:
 - Recently hospitalized patient's evaluation within 1 week of discharge[N]
 - Recently hospitalized patient's evaluation with outpatient healthcare providers (eg, nurse, social worker) within 1 to 3 days of discharge(3)(4)(5)
 - Appointment every 3 to 6 months when patient is symptom free
 - Appointment once a month or more for patient with questionable adherence to treatment plan
- **EDUCATE:** When and how to get help for worsening schizophrenia problems
- **EDUCATE:** Treatment and monitoring for comorbid conditions (eg, coronary artery disease, high blood pressure, heart valve disease)(6)
- **EDUCATE:** Follow-up appointments for assessment of clinical status
- **COORDINATE:** Specialist referral, if indicated
- **COORDINATE:** Care team meeting, if needed(7)(8)
- **COORDINATE:** Follow-up to review progress with self-care plan, referral status, and need for further assistance(3)(9)
- **RECONCILE:** Communicate care plan changes to entire care team.(10)(11)

P2. Schizophrenia self-care information needed

- **Goal:** Schizophrenia self-care information understood(12)[NNN]
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Support system (eg, family, friends) identification, enlistment, and education
 - **ASSIST:** Medical alert bracelet/necklace, obtain and wear
 - **EDUCATE:** Schizophrenia is a complex brain disease that results in impaired thoughts, perceptions, cognitive functions, moods, and motivations.(13)(14)
 - Often called a psychotic disorder because individuals lose touch with reality and exhibit psychotic symptoms (eg, hallucinations, delusions, or catatonia)
 - Causes profound disruption in lives of those diagnosed and the people around them
 - Lifelong debilitating illness with no cure
 - **EDUCATE: Self-care plan information(6):**
 - Action plan (eg, when to initiate relaxation, cognitive, and stress-management techniques)
 - Adherence to plan necessary to achieve goals, and complications of nonadherence (eg, exacerbations, progression, comorbidities)[B](14)
 - Daily care (eg, keep a routine, good hygiene, take medicine)
 - Medication purpose, use, effects, and considerations (eg, timing before meals or exercise)(15)
 - Caregiver support plan, as needed
 - Crisis care plan: turn off distractions (eg, TV, radio), seek emergent care, call hotline, call police if escalates; caregivers advised to not argue(13)(16)
 - Set small goals every day (eg, get out of bed at a certain time, finish one chore or project, or do something that brings pleasure like listen to music).(6)(13)
 - Health checks, diary, and action plans (eg, write down thoughts, feelings, daily activities, eating times, and when medication taken)(13)
 - Diet considerations (eg, limit caffeine, avoid alcohol)
 - Exercise program within provider's recommendations(17)(18)(19)[NN]
 - Sleep hygiene (eg, regular pattern, calm sleep environment)
 - Functional changes care (eg, adaptations and support for ADL and IADL)
 - Self-care plan guided adjustment, if indicated
 - Auditory hallucination care techniques (eg, use headphones with music, talk to others)
 - Behavioral health treatment (eg, cognitive behavioral therapy (CBT), individual therapy, family therapy)[C](13)(20)[NNN]
 - Trigger avoidance (eg, high-stress situations, sleep deprivation)(14)
 - Relapse signs (eg, insomnia, decreased appetite, poor personal hygiene)(14)
 - Support group participation
 - Safety awareness and management (eg, remove suicide risks, de-escalation techniques)
 - Wellness checkups (eg, vaccinations, screenings)
 - Substance use avoidance[D](13)(20)(21)[N]
 - Tobacco cessation plan(15)[NN]
 - **EDUCATE:** Complications with schizophrenia include:
 - Confusion

- Delusions
- Hallucinations
- Relapse(20)
- Trouble completing activities
- Sleeping problems
- **EDUCATE: When to get emergency help(16)(22):**
 - Suicidal or homicidal actions or plans(14)
 - Heart attack (eg, persistent chest pain; pain radiating to the arms, shoulders, neck, back, and/or jaw; cool clammy skin)(23)
 - Stroke (eg, face drooping, arm weakness, speech problems, bad headache, vision changes, loss of balance or coordination)(24)
 - Crisis plan not working
 - Psychotic behavior (eg, threatening)
 - Respiratory impairment (eg, gasping for air, choking, cyanosis, chest pain, severe dyspnea)(25)
 - Fatigue, extreme
 - Hallucinations or delusions, increased or new
 - Delirium symptoms (eg, reduced awareness, agitation)
 - Seizures, new or complicated (eg, seizure longer than 5 minutes, multiple seizures with loss of consciousness, head injury during seizure)(26)
 - Bowel or bladder function loss
- **EDUCATE: When to call the provider for behavior changes(6)(14):**
 - Appointments missed (eg, support group, provider)
 - Cognitive changes (eg, thinking problems, forgetfulness worsens)
 - Disorganization of thoughts (eg, flight of ideas, rapidly switching subjects in conversation)
 - Hallucinations or delusions increase.
 - Mood changes (eg, depression, anger, agitation)
 - Self-care changes (eg, ADL or IADL problems)
 - Self-harm (eg, abuse, mutilation)
 - Social isolation
 - Substance misuse (eg, increased use, use interferes with ADL/IADL)
 - Suspiciousness or paranoia increase
 - Life event, major (eg, job loss, death of someone close)
- **EDUCATE: When to call the provider(6)(14):**
 - Weight gain or loss without reason
 - Constipation
 - Medication problems (eg, side effects, access, administration)
 - Medications newly ordered from other providers
 - Pregnant or planning to get pregnant(27)(28)
- **COORDINATE:** Behavioral health services referral, if indicated(13)
- **COORDINATE:** Psychiatrist referral, if indicated(13)
- **COORDINATE:** Chronic condition management referral, if indicated(29)
- **COORDINATE:** Community services referral, as needed(30)
- **COORDINATE:** Crisis services referral, as needed
- **COORDINATE:** Outpatient imaging, tests, or procedures, if indicated
- **COORDINATE:** Home care referral, if indicated(10) 
- **COORDINATE:** Telehealth referral, if indicated[E]
- **COORDINATE:** Social worker referral, as needed(31)
- **COORDINATE:** Support group referral, as needed(30)
- **COORDINATE:** Treatment program referral, if indicated(13)
- **COORDINATE:** Tobacco use treatment, as needed
- **COORDINATE:** Vocational rehabilitation referral, as needed(15)
- **COORDINATE:** Follow-up contact with patient to evaluate understanding and adherence to self-care plan, referral status, and need for further assistance(3)(9)
- **RECONCILE:** Self-care plans from involved care team members
- **RECONCILE:** Communicate care plan changes to entire care team.(10)(11)
- **SEND:** What you need to know - Schizophrenia  (Spanish)
- **SEND:** Your care plan - Schizophrenia  (Spanish)
- **SEND:** Daily care for your condition - Schizophrenia  (Spanish)
- **SEND:** When to get help - Schizophrenia  (Spanish)
- **SEND:** Be ready for your doctor visit - Schizophrenia  (Spanish)

- **SEND:** Be ready with questions for your doctor - Schizophrenia 📄 (Spanish)
- **SEND:** Tips for good mental health 📄 (Spanish)
- **SEND:** Caregiver tips 📄 (Spanish)
- **SEND:** Choosing a place to live 📄 (Spanish)
- **SEND:** Help with sleeping or insomnia 📄 (Spanish)
- **SEND:** How to stop smoking 📄 (Spanish)
- **SEND:** Symptoms diary (Illustrated) 📄 (Spanish)
- **SEND:** Coronavirus disease 2019 (COVID-19) vaccines 📄 (Spanish)
- **SEND:** Flu vaccine 📄 (Spanish)

P3. Unhealthy behaviors potentially causing complications with treatment plan

- **Goal:** Information, support, and reinforcement of healthy behaviors to facilitate treatment plan understood(32)
 - **Interventions:**
 - **RECONCILE:** Treatment plan with patient's willingness and ability to facilitate in presence of unhealthy behaviors
 - **ASSIST:** Identifying types of unhealthy behaviors patients may exhibit, such as(33):
 - Aggressiveness, hostility, or abusiveness
 - Intentional nonadherence
 - Ineffective coping (eg, denial, poor insight, unmanageable anger, low self-esteem)
 - Self-care neglect (eg, not bathing, dressing, eating)
 - Social isolation or withdrawal
 - **COORDINATE:** Communication to care team members regarding potential problems with treatment plan related to patient's unhealthy behaviors
 - **ASSIST:** Appointment scheduling with care providers if unhealthy behaviors causing problems with patient's treatment plan
 - **ASSIST:** Creating behavior support plan to help facilitate treatment, including:
 - Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - Identifying factors that contribute to unhealthy behaviors and alternative behaviors that may help patient manage
 - Identifying support people who can help patient with behavior changes
 - Setting limits on patient's behavior and withdrawing attention from patient if behavior presents
 - Creation of emergency action plan if patient's unhealthy behavior escalates (eg, patient risk for harm to self or others)(34)
 - Providing positive reinforcement of healthy behaviors
 - **EDUCATE:** Unhealthy behaviors and potential consequences for treatment or outcomes
 - **EDUCATE:** Goals to help overcome unhealthy behaviors, including interventions, steps, and time frames
 - **EDUCATE:** Alternative coping mechanisms to help patient with behavior changes, such as(35)(36):
 - Social participation (eg, community groups, social organizations, family gatherings)
 - Recreation or hobbies participation
 - Behavioral health support (eg, counseling), role and benefit
 - Relaxation techniques (eg, deep breathing, guided imagery, music)
 - Stress management (eg, relaxation and coping techniques)
 - **EDUCATE:** When and how to get help for problems(37)
 - **EDUCATE:** Caregivers on managing unhealthy behaviors(35)(38):
 - Provide positive reinforcement for healthy behaviors.
 - Be aware of caregiver's personal feelings and their impact on patient's behavior.
 - Support the patient's self-responsibility (eg, independence with hygiene, taking medications, decision making).
 - Know signs of escalating behavior problems (eg, risk for harm to self or others) and when and how to get help.
 - Use agitation reduction techniques (eg, avoid overstimulation, provide privacy, encourage activities, learn to de-escalate).(39)
 - De-escalation techniques (eg, caregiver remains calm and provides emotional support, do not argue or threaten, stay serious, announce actions beforehand)(22)(33)
 - **COORDINATE:** Behavioral health services referral, if indicated
 - **COORDINATE:** Psychiatrist referral, if indicated
 - **COORDINATE:** Recreational therapy referral, if indicated
 - **COORDINATE:** Protective services or legal authorities, as needed
 - **COORDINATE:** Follow-up to evaluate status with behavior changes, treatment plan, referrals, and need for further assistance(3)(9)
 - **SEND:** Tips for good mental health 📄 (Spanish)
 - **SEND:** Caregiver tips 📄 (Spanish)

P4. Coordination of care needed for schizophrenia patient

- **Goal:** Self-care plan for schizophrenia communicated to all care team members(10)(12)
 - **Interventions:**
 - **RECONCILE:** Self-care plans from involved care team members
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Scheduling and preparing for provider appointments
 - **ASSIST:** Creation of a self-care plan between patient and providers to include(40)(41):
 - Identifying patient's self-care goals
 - Establishing daily management (medications, diet changes, health checks, and general wellness)
 - Creating healthcare diary to track self-care information (eg, vital signs) and to keep written record of care plan changes (eg, changes in medication and when laboratory work is due)
 - **ASSIST:** Communication between care team members
 - **EDUCATE:** Care team members, contact information, and role in care planning
 - **EDUCATE:** Self-care plan and when, where, and from whom to get help for problems with care(8)
 - **COORDINATE:** Meeting with care team members, as needed
 - **COORDINATE:** Behavioral health services referral, if indicated
 - **COORDINATE:** Home care referral, if indicated
 - **COORDINATE:** Psychiatrist referral, if indicated
 - **COORDINATE:** Rehabilitation therapy referral, if indicated
 - **COORDINATE:** Community services referral, as needed
 - **COORDINATE:** Social worker referral, as needed
 - **COORDINATE:** Durable medical equipment referral, if indicated
 - **COORDINATE:** Follow-up contact with patient to evaluate understanding and adherence to self-care plan, referral status, and need for further assistance(3)(9)
 - **COORDINATE:** Chronic condition management referral, if indicated(29)
 - **COORDINATE:** Transmission of handoff communication
 - **RECONCILE:** Communicate care plan changes to entire care team.(11)
 - **SEND:** Your care plan - Schizophrenia 📄 (Spanish)
 - **SEND:** What you need to know - Schizophrenia 📄 (Spanish)
 - **SEND:** Daily care for your condition - Schizophrenia 📄 (Spanish)
 - **SEND:** When to get help - Schizophrenia 📄 (Spanish)
 - **SEND:** Be ready for your doctor visit - Schizophrenia 📄 (Spanish)
 - **SEND:** Be ready with questions for your doctor - Schizophrenia 📄 (Spanish)

P5. Potential complication: financial issues present

- **Goal:** Financial resources identified and sufficient to continue care(42)(43) 📄
- **Interventions:**
 - **ASSIST:** Identifying healthcare benefits coverage from payers
 - **ASSIST:** Identifying availability of and eligibility for alternative financial resources
 - **ASSIST:** Communicating information on patient's healthcare benefits coverage with treatment team
 - **ASSIST:** Appointment scheduling and completing paperwork to support applications for financial support
 - **EDUCATE:** Healthcare benefits and out-of-pocket expenses
 - **EDUCATE:** Resources for financial assistance:
 - Administration for Children & Families: www.acf.hhs.gov
 - Centers for Medicare and Medicaid Services (CMS): www.medicare.gov/drug-coverage-part-d
 - Medicine Assistance Tool: <https://medicineassistancetool.org>
 - NeedyMeds. Find help with the cost of medicine: www.needymeds.org
 - Individual pharmaceutical company websites
 - **COORDINATE:** Financial assistance referral; examples include:
 - Area Agency on Aging
 - Disease advocacy
 - Financial counseling or assistance
 - Food banks or other sources for nutritional support and financial assistance
 - Government agencies
 - Legal aid
 - Long-term disability application
 - Medicaid application
 - Medicare application

- Pharmacy assistance programs
- Short-term disability application
- Supplemental Security Income (SSI) or disability benefits application
- Veterans Affairs (VA) benefits application
- **COORDINATE:** Social worker referral, as needed(31)
- **COORDINATE:** Follow-up for evaluation of referral status and need for further assistance(3)(11)
- **Goal:** Financial issues addressed(42)
 - **Interventions:**
 - **RECONCILE:** Benefits from multiple sources, if needed
 - **ASSIST:** Obtaining preauthorization from health plan provider, if appropriate
 - **EDUCATE:** Source and type of financial support, how to access, and if follow-up needed
 - **COORDINATE:** Follow-up for evaluation of use of support programs or ongoing need for further assistance(3)(11)

P6. Potential complication: self-care needs exceed current ability.

- **Goal:** Self-care needs safely met during and after care transition(5)(44)
- **Interventions:**
 - **RECONCILE:** Patient's self-care needs and ability with transition plan; considerations include:
 - Complexity of self-care needs
 - Patient's preferences for next level of care
 - Care recommendations from care team members
 - Available and able caregiver at next level of care
 - Financial resources and out-of-pocket expenses
 - **ASSIST:** Identifying discharge options that will meet patient's self-care needs
 - **EDUCATE:** Care team's recommendation for next level of care, including:
 - Self-care needs and goals for care at next level of care
 - Role and purpose of resources at next level of care
 - Anticipated capability of support needed at next level of care (eg, skilled, trained)
 - Estimated time (duration) needed for support at next level of care
 - **EDUCATE:** Providers of care available to patient for transition (eg, list of facilities or providers)
 - **EDUCATE:** Options for alternative levels of care that may also support patient self-care
 - **EDUCATE:** Potential complications with proceeding with discharge plan that does not cover patient's self-care needs
 - **EDUCATE:** Financial resources and out-of-pocket expenses for various care levels
 - **COORDINATE:** Social worker referral, as needed(31)
 - **COORDINATE:** Home care referral, if indicated
 - **COORDINATE:** Community services referral, as needed
 - **COORDINATE:** Communication between care team members, patient's caregivers, alternative level-of-care providers, and financial resources (health plan)
 - **COORDINATE:** Follow-up contact to evaluate patient's discharge preference, referral status, and need for further assistance

P7. Potential complication: caregiver needed to support patient's self-care

- **Goal:** Caregiver support available and provided for safe transition to next level of care(38)(45)
- **Interventions:**
 - **ASSIST:** Support system (eg, family, friends) identification, enlistment, and education
 - **ASSIST:** Deploying strategies to reduce caregiver burden, as needed (eg, healthcare benefits, support groups, family leave)(46)
 - **EDUCATE:** Patient's self-care needs and goals for care at next level of care
 - **EDUCATE:** When and how to get help for problems with patient or patient care
 - **EDUCATE:** Caregiver's anticipated role (eg, assisting with ADL, performing treatments, managing appointments)
 - **EDUCATE:** Risk to long-term caregiver for depression, anxiety, sleep disturbance, and burnout(47)(48)
 - **EDUCATE:** Caregiver support(46):
 - Identification of other caregivers
 - Consideration of adult day care or respite program
 - Self-care needs (eg, medical, behavior health, social support)
 - **COORDINATE:** Care team meeting, if needed
 - **COORDINATE:** Educational program referral for caregiver
 - **COORDINATE:** Social worker referral, as needed
 - **COORDINATE:** Community services referral, as needed
 - **COORDINATE:** Follow-up contact to review caregiver resource needs, referral status, and need for additional support
 - **SEND:** Caregiver tips  (Spanish)

P8. Medication information needed

- **Goal:** Medication need, use, and management evaluated and problems resolved(44)(49)

- **Interventions:**

- **RECONCILE:** Medication list. See Medication Reconciliation Tool  SR for more information. 
- **ASSIST:** Creation of current and reconciled medication list
- **ASSIST:** Providing reconciled medication list with all providers. See Medication Reconciliation Tool  SR for more information.
- **ASSIST:** Identifying resources to pay for medications, if needed
- **ASSIST:** Identifying activities that promote medication adherence
- **ASSIST:** Communication with treating provider when another provider (eg, emergency or specialty provider) prescribes new medication
- **ASSIST:** Appointment scheduling with provider to evaluate treatment outcomes
- **EDUCATE: Reason for taking medication(50):**
 - Expected effects if the medication is working, and expected time for effects to occur
 - How long medication will likely need to be taken
 - Risks of nonadherence
- **EDUCATE:** Information on all medications(49)(50)(51)(52):
 - Medication purpose, dose, effects/side effects, and considerations (eg, managing missed doses, interactions with food/other drugs)
 - Name of provider who ordered medication
 - Administration techniques
 - Side effects, related management, and when to contact provider
 - **Get emergency help:** Adverse or allergic reactions (eg, wheezing, chest tightness, fever, itching, bad cough, blue skin color, hemorrhage, seizures, swelling of face, lips, tongue, or throat)
 - Common side effects include gastrointestinal changes, rash, dizziness, and headache.
 - Do not stop long-term medications abruptly due to risk of withdrawal symptoms.
 - Medication safety (eg, proper storage and disposal)(53)
 - Monitoring (eg, vital signs, laboratory checks) needed while taking medication
 - Talk to provider before taking any over-the-counter medications, vitamins, minerals, or herbal or dietary supplements.
 - Carry a list of medications at all times, particularly when visiting healthcare providers.(52)
- **EDUCATE:** Ways to improve following medication routine (eg, pillboxes, alarm reminders)
- **EDUCATE:** Pharmacy assistance programs, such as:
 - Centers for Medicare and Medicaid Services (CMS): www.medicare.gov/drug-coverage-part-d
 - Medicine Assistance Tool: <https://medicineassistancetool.org>
 - NeedyMeds: www.needymeds.org
 - Individual company websites
- **COORDINATE:** Follow-up on the medication plan:
 - Contact with prescribing providers to verify medication orders
 - Contact with pharmacy to confirm patient receipt of medication
 - Contact with patient to evaluate adherence to medication plan and need for more assistance
 - Laboratory testing, if indicated(54)
- **COORDINATE:** Home care referral, if indicated
- **COORDINATE:** Follow-up to evaluate adherence or problems with medication, provide additional education and support, and identify need for further assistance(3)(9)
- **RECONCILE:** Communicate reconciled medication list to all providers.
- **SEND:** Managing your medicines (Illustrated)  (Spanish)

- **Goal:** Medication safety understood and medication safety implemented(49)

- **Interventions:**

- **RECONCILE:** Medication list. See Medication Reconciliation Tool  SR for more information.
- **ASSIST:** Communication with treating provider when another provider (eg, emergency or specialty provider) prescribes new medication
- **ASSIST:** Communication with primary provider about prescription issues:
 - Polypharmacy
 - Food-drug and drug-drug issues
 - Adherence problems
- **EDUCATE:** Medication safety instructions(55)(56):
 - Discard outdated medication.
 - Do not share prescription medication.
 - Store out of reach of children and in childproof containers.
 - Fill all prescriptions at the same pharmacy.
 - Store properly (in original containers and with other needs such as refrigeration or protection from sunlight).

- Check with provider before taking over-the-counter medications, vitamins, minerals, or herbal or dietary supplements.
- How to dispose of unused or expired medication
- **EDUCATE:** Administration techniques
- **EDUCATE:** De-prescribing or cessation of medications safely (eg, pharmacy-supported, provider monitoring planned) (57)
- **COORDINATE:** Home care referral, if indicated
- **COORDINATE:** Follow-up to evaluate adherence or problems with medication, provide additional education and support, evaluate referral status, and identify need for further assistance(3)(9)
- **RECONCILE:** Communicate reconciled medication list to all providers.
- **SEND:** Managing your medicines (Illustrated)  (Spanish)

P9. Potential complication: transportation insufficient for patient to access care

- **Goal:** Transportation issues addressed(5)(58) 
- **Interventions:**
 - **ASSIST:** Identifying transportation resources and accessing as needed, such as:
 - Family, friends, neighbors, community group members
 - Public transportation (bus, train, access vans)
 - Private transportation (taxis, car service)
 - Pharmacy, equipment, or supply delivery services
 - **ASSIST:** Completing transportation assistance forms, as needed
 - **EDUCATE:** Local transportation services available and how to access them
 - **EDUCATE:** Cost associated with available transportation
 - **EDUCATE:** When and how to get help if transportation continues to be a problem
 - **COORDINATE:** Communication between care team members on how patient is transported to and from service visits
 - **COORDINATE:** Community services referral, as needed
 - **COORDINATE:** Social worker referral, as needed(31)
 - **COORDINATE:** Follow-up contact to evaluate referral status, use of transportation, and address any further transportation needs

P10. Schizophrenia medication management needed

- **Goal:** Medication information understood(52)
- **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for discussion on appropriate medication
 - **ASSIST:** Providing reconciled medication list to all providers. See Medication Reconciliation Tool  SR for more information.
 - **EDUCATE: Purpose:** Importance of understanding what each medication is for, how it should be taken, and what to do if dose is missed
 - **EDUCATE:** Medication indications, side effects, and considerations frequently change. See a drug reference for specific information not listed here.
 - **EDUCATE:** Administration techniques:
 - Route (eg, oral, intramuscular (IM), intravenous (IV))
 - With or without foods or liquids (eg, milk)
 - Timing of administration
 - Appropriate measuring device (eg, liquid medications)
 - Proper storage
 - **EDUCATE: Get emergency help:** Adverse or allergic reactions (eg, wheezing, chest tightness, fever, itching, bad cough, blue skin color, hemorrhage, seizures, swelling of face, lips, tongue, or throat)
 - **EDUCATE: Side effects:** Management, and when to contact provider
 - Common side effects include gastrointestinal changes, dry mouth, fatigue, insomnia, rash, dizziness, and headache.
 - **EDUCATE: Considerations:**
 - Discuss medication with provider if pregnant, planning a pregnancy, or breast-feeding.
 - Comorbidities may be contraindications to the use of a drug (eg, liver disease, diabetes).
 - Health checks, diary, and action plans (eg, blood pressure, weight, heart rate)
 - Safety measures (eg, sun protection, driving, or operating machinery)
 - Contact provider or pharmacist prior to taking any other over-the-counter or prescribed medication.
 - **COORDINATE:** Clinical monitoring appropriate to medication use (eg, laboratory tests)

- **COORDINATE:** Communication between patient and pharmacy (for obtaining prescriptions, setting up home medication delivery, review for drug-drug interactions)^[N]
- **COORDINATE:** Communication between care team members and medication list reconciliation
- **COORDINATE:** Follow-up to evaluate adherence or problems with medication, provide additional education and support, and identify need for further assistance⁽³⁾⁽⁹⁾
- **COORDINATE:** Laboratory testing, if indicated
- **COORDINATE:** Outpatient imaging, tests, or procedures, if indicated
- **COORDINATE:** Therapeutic drug level monitoring
- **SEND:** Managing your medicines (Illustrated)  (Spanish)

P11. Atypical antipsychotic medication management needed

- **Goal:** Atypical antipsychotic medication initiated and maintained
 - **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** Atypical antipsychotics may be indicated for rapid control of acute mania to decrease agitation, aggression, and dangerous behavior.⁽⁵⁹⁾⁽⁶⁰⁾
 - Second-generation antipsychotic medication
 - **EDUCATE: Call provider immediately:** Neuroleptic malignant syndrome (NMS) (eg, extreme muscle rigidity, high fever, tachycardia, tachypnea, diaphoresis, stupor, fluctuations in blood pressure)^{[F](61)}
 - **EDUCATE: Side effects:** Difficulty urinating; jaundice; shuffling walk; fainting spells; seizures; pharyngitis; muscle spasms; excitement or restlessness; jerky movements of head, face, or neck; tachycardia; weight gain; blurred vision⁽⁶²⁾
 - Increased risk of metabolic side effects, including obesity, diabetes, and dyslipidemia⁽⁶⁰⁾
 - **EDUCATE: Considerations**⁽⁶⁰⁾:
 - Use cautiously with history of seizures, liver disease, heart disease, blood disorders, or pregnancy.⁽⁶³⁾
 - Monitor blood glucose, cholesterol, and triglyceride levels.
 - Talk to provider before discontinuing medication.
 - **COORDINATE:** Electrocardiogram (ECG) monitoring, if indicated⁽⁶⁰⁾
 - **COORDINATE:** Laboratory testing

P12. Typical antipsychotic medication management needed

- **Goal:** Typical antipsychotic medication initiated and maintained⁽⁶⁰⁾
 - **Interventions:**
 - **EDUCATE: Purpose:** Antipsychotics are used to treat hallucinations, delusions, disorganized thoughts and behavior, and related agitation/aggression.
 - Long-acting injectable (LAI) antipsychotics may improve adherence.
 - **EDUCATE: Call provider immediately:** Neuroleptic malignant syndrome (NMS) (eg, extreme muscle rigidity, high fever, tachycardia, tachypnea, diaphoresis, stupor, fluctuations in blood pressure)^{[F](61)}
 - **EDUCATE: Side effects:** Drowsiness, dry mouth, weight gain, ECG changes, involuntary reflexes and movements, hyperglycemia, diabetes
 - Tardive dyskinesia (involuntary repetitive movements, such as grimacing, tongue movements, lip smacking, pursed lips, excessive eye blinking) can occur.^[G]
 - Increased risk of metabolic side effects, including obesity, diabetes, and dyslipidemia
 - **EDUCATE: Considerations:**
 - PRN medications may be ordered to address involuntary reflexes and movements.
 - Monitor blood glucose, cholesterol, and triglyceride levels.
 - Diet and exercise instruction
 - Modest efficacy and increases risk of cognitive decline and cerebrovascular events^{[H](63)}
 - Talk to provider before discontinuing medication.
 - **COORDINATE:** Electrocardiogram (ECG) monitoring, if indicated

P13. Schizophrenia medication evaluation needed

- **Goal:** Schizophrenia medication evaluation completed and regimen established
 - **Interventions:**
 - **RECONCILE:** Medication list. See Medication Reconciliation Tool  SR for more information.
 - **ASSIST:** Appointment scheduling with prescribing provider for evaluation of condition status
 - **EDUCATE:** When and how to get help for worsening schizophrenia problems
 - **EDUCATE:** Medication management needs common for patients with schizophrenia
 - **COORDINATE:** Follow-up after provider contact for evaluation of medication changes
 - **RECONCILE:** Communicate reconciled medication list to all providers.⁽⁶⁴⁾⁽⁶⁵⁾

P14. Medication changes present

- **Goal:** Medication changes understood and medication taken as ordered

- o **Interventions:**

- **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
- **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
- **ASSIST:** Appointment scheduling with provider for evaluation of medication problems (clarification of dose or regimen)
- **ASSIST:** Creation of current and reconciled medication list
- **ASSIST:** Identifying activities that promote medication adherence
- **ASSIST:** Communication with treating provider when another provider (eg, emergency or specialty provider) prescribes new medication
- **EDUCATE: Reason for taking medication(50):**
 - Expected effects if the medication is working, and expected time for effects to occur
 - How long medication will likely need to be taken
 - Risks of nonadherence
- **EDUCATE: Information on all medications(49)(50)(51)(52):**
 - Medication purpose, dose, effects/side effects, and considerations (eg, managing missed doses, interactions with food/other drugs)
 - Name of provider who ordered medication
 - Administration techniques
 - Side effects, related management, and when to contact provider
 - **Get emergency help:** Adverse or allergic reactions (eg, wheezing, chest tightness, fever, itching, bad cough, blue skin color, hemorrhage, seizures, swelling of face, lips, tongue, or throat)
 - Common side effects include gastrointestinal changes, rash, dizziness, and headache.
 - Do not stop long-term medications abruptly due to risk of withdrawal symptoms.
 - Medication safety (eg, proper storage and disposal)(53)
 - Monitoring (eg, vital signs, laboratory checks) needed while taking medication
 - Talk to provider before taking any over-the-counter medications, vitamins, minerals, or herbal or dietary supplements.
 - Carry a list of medications at all times, particularly when visiting healthcare providers.
- **COORDINATE:** Home care referral, if indicated
- **COORDINATE:** Communication between care team members and medication list reconciliation.(49)(64) See Medication Reconciliation Tool  SR for more information.
- **COORDINATE:** Communication between patient and pharmacy (for obtaining prescriptions, setting up home medication delivery, review for drug-drug interactions)(66)
- **COORDINATE:** Follow-up contact for evaluation of medication understanding, referral status, and need for further assistance(3)(9)
- **SEND:** Managing your medicines (Illustrated)  (Spanish)

P15. Medication refills needed

- **Goal:** Medications refilled

- o **Interventions:**

- **RECONCILE:** Medication list. See Medication Reconciliation Tool  SR for more information.
- **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
- **ASSIST:** Obtaining refills in lieu of an appointment
- **ASSIST:** Identifying resources to pay for medications, if needed
- **ASSIST:** Working with pharmacy for filling prescriptions (eg, changing pills to liquids, identifying financial support, delivery)
- **COORDINATE:** Prescription refill support (eg, pharmacy programs) referral
- **COORDINATE:** Social worker referral, as needed
- **EDUCATE:** Refill medications prior to running out (1 to 2 weeks before).
- **EDUCATE:** Pharmacy assistance programs, such as:
 - Centers for Medicare and Medicaid Services (CMS): www.medicare.gov/drug-coverage-part-d
 - Medicine Assistance Tool: <https://medicineassistancetool.org>
 - NeedyMeds: www.needymeds.org
 - Individual company websites
- **COORDINATE:** Follow-up to evaluate continued need for refills, referral status, and need for further assistance(3)(9)
- **RECONCILE:** Communicate reconciled medication list to all providers.(64)(65)
- **SEND:** Managing your medicines (Illustrated)  (Spanish)

P16. Laboratory work coordination needed

- **Goal:** Laboratory work completed(44)

- o **Interventions:**

- **RECONCILE:** Laboratory work needed from all providers
- **COORDINATE:** Laboratory testing, if indicated
- **ASSIST:** Identifying laboratory location and scheduling appointment, if needed
- **ASSIST:** Identifying transportation resources and accessing as needed
- **EDUCATE:** Purpose and importance of getting laboratory work
- **EDUCATE:** Follow-up with provider for laboratory results
- **COORDINATE:** Follow-up contact with provider after laboratory work done to evaluate patient's status
- **COORDINATE:** Communicate laboratory results to all relevant providers.
- **COORDINATE:** Follow-up to review laboratory results and follow-up care needs, and to evaluate referral status or need for further assistance(3)(9)
- **SEND:** Reminder on laboratory work due date, location, and provider follow-up

P17. Potential complication with transition to post-acute care facility

- **Goal:** Potential complication with transition to post-acute care facility resolved(5)(44)

- **Interventions:**

- **RECONCILE:** Transition care plan (eg, preferences, abilities, resources)(67)
- **COORDINATE:** Care team meeting, if needed(7)(8)
- **ASSIST:** Identifying 3 facilities within reconciled level-of-care options:
 - Identifying facilities in or out of health plan network, if indicated
 - Site visit to facilities by family or healthcare proxy
- **ASSIST:** Completion of admission paperwork by patient or designee
- **COORDINATE:** Preauthorization with health plan(s), if indicated
- **COORDINATE:** Post-acute care facility pre-transition evaluation, if indicated
- **EDUCATE:** Level-of-care options meeting care team recommendations and within payment considerations
- **EDUCATE:** Benefits for this level of care and any out-of-pocket expenses that may be patient's responsibility (eg, co-pays or deductibles)
- **EDUCATE:** Post-acute care facility information (name, contact, and transfer date)
- **EDUCATE:** Goals and expectations once transition complete
- **EDUCATE:** Transition of care plan (including estimated timing of events)
- **SEND:** Medical records to accepting facility:
 - History and physical
 - Transition of care summary
 - Last set of laboratory results
 - Discharge orders (including treatments, laboratory tests, medications)
 - Opioid prescription signed hard copy, if indicated
 - Results of tests or procedures
 - Information on consulting provider(s) and when follow-up is needed
 - Advance care planning information
- **SEND:** Transition summary to primary care provider
- **COORDINATE:** Confirmation of facility acceptance, confirming:
 - Bed availability
 - Admission date and time
 - Transport of patient to facility
 - Medical record and discharge orders received
- **COORDINATE:** Disposition of patient's personal items
- **COORDINATE:** Social worker referral, as needed(31)
- **COORDINATE:** Follow-up contact with facility or provider, if needed
- **COORDINATE:** Follow-up contact to evaluate for transition problems, referral status, and need for further assistance(3)(11)
- **RECONCILE:** Timing of care transition and self-care plan (eg, medication available, provider assessment)
- **RECONCILE:** Discharge and admission orders
- **SEND:** When you are going to a recovery facility for care  (Spanish)

- **Goal:** Alternative care transition plan initiated if complication with post-acute facility transition cannot be resolved(44)(68)

- **Interventions:**

- **RECONCILE:** Patient (or designee) preferences, care team recommendations, and payment options for alternative care transition plan
- **ASSIST:** Identifying alternative transition plans
- **ASSIST:** Identifying resources to support alternative care transition plan
- **EDUCATE:** Alternative levels of care besides post-acute care facility that will meet patient's needs
- **EDUCATE:** Why post-acute care facility may be insufficient to meet patient's needs
- **COORDINATE:** Care team meeting to discuss alternative care transition plan options(7)(8)
- **COORDINATE:** Outpatient imaging, tests, or procedures, if indicated

- **COORDINATE:** Home care referral, if indicated
- **COORDINATE:** Community services referral, as needed
- **COORDINATE:** Inpatient rehabilitation facility referral, if indicated
- **COORDINATE:** End-of-life or palliative care referral, if indicated
- **COORDINATE:** Assisted living referral, if indicated
- **COORDINATE:** Long-term care services and support referral, as needed
- **COORDINATE:** Follow-up contact to evaluate alternative discharge options, referral status, and need for further assistance(3)(11)

Evidence Summary

Quality standards: Standards indicate that programs should ensure that during care transitions the care plan is re-evaluated, modified, and communicated to appropriate care team members (eg, providers, caregivers) as needed to support the care transition.(10)(69)(70)(71)(72)

Quality standards: The American Case Management Association's Transitions of Care Standards indicate that patients' health risks should be assessed at each care episode or transition, a care management plan should be established and communicated across the care settings, and an evaluation should be done of the patient/family/caregiver's understanding of health status and self-care.(73)

Quality standards: The American Case Management Association's Transitions of Care Standards indicate that processes should be in place to identify patients at risk for care transition problems, patients' health risks should be assessed at each care episode or transition, and a care management plan should be established and communicated across care settings.(73)

Transition consideration: Several studies demonstrated a reduction in 30-day hospital readmissions when the patient follow-up was within 7 days of hospital discharge.(74)(75)(76) Earlier follow-up (eg, within 48 hours after hospital discharge) may also help reduce hospital readmissions for high-risk or vulnerable patients.(3) Follow-up appointment attendance rates were higher when appointments were scheduled before hospital discharge.(77)(78)

Quality standards: Standards indicate that programs should develop and communicate an individualized case management plan with the patient, facilitate and follow up with referrals, and assess the patient's progress with the care plan. Programs should monitor and communicate the patient's progress with the care plan, modifying the plan as needed, and continue to identify patient preferences, barriers, and adherence concerns. Program content should include an individualized self-care plan that includes patient preferences, prioritized goals, interventions with information on whom to contact in urgent situations, and scheduled follow-up. Programs should facilitate communication and collaboration with practitioners in a timely manner.(69)(71)(72)

Quality standards: The Case Management Society of America's Standards of Practice for Case Management principles include facilitation of informed decision making, counseling, and health education with patient self-care that is responsive to the patient's preferences (eg, culture, values, needs).(69)

Quality standards: The Case Management Society of America's Case Management Adherence Guide includes guidance for improving adherence via patient engagement.(79)

National guidelines: The 2018 Physical Activity Guidelines Advisory Committee advises that adults with chronic conditions or disabilities, who are able, should follow the key activity guidelines for adults. The guidelines recommend that adults should do at least 150 to 300 minutes a week of moderate-intensity or 75 to 150 minutes a week of vigorous-intensity aerobic physical activity, or an equivalent combination of moderate-intensity and vigorous-intensity aerobic activity. They should also do muscle-strengthening activities on 2 or more days a week. Older adults should do multicomponent physical activity that includes balance training as well as aerobic and muscle-strengthening activities.(80)

Specialty society: The National Institute for Health and Care Excellence (NICE) recommends offering combined healthy eating and physical activity programs to help reduce the increased rates of obesity and type 2 diabetes found in adults with psychosis or schizophrenia.(81)

National guidelines: The National Institute for Health and Care Excellence (NICE) guidelines suggest that all patients (adults and children) with psychosis or schizophrenia, including those with acute or recurrent symptoms, receive family therapy (if applicable) as an adjunct to individual cognitive behavioral therapy (CBT).(15)

Evidence: Randomized controlled trials and meta-analyses have found that when psychosocial care (eg, family-based intervention) is integrated with standard pharmacologic care, there is a reduced risk of relapse and hospitalization.(15)(82)

Evidence: A review of 24 studies involving 1900 participants suggests that group cognitive behavioral therapy (CBT) may be better than standard care or other psychosocial interventions for overall mental state and global functioning for people with schizophrenia.(83)

Evidence: Studies suggest that having ever used cannabis was associated with a modest increased risk of psychotic disorders, daily use was associated with a higher risk, and daily use of high-potency cannabis conferred a risk of more than 4 times that of ever having

used cannabis.(20)

Specialty society: The American Psychiatric Association states that smoking is a major contributor to increased mortality in individuals with serious mental illness and advises that health education and motivational interviewing may be helpful in those who have had prior unsuccessful attempts at smoking cessation.(84)

Specialty society: Smoking tobacco or marijuana reduces the serum levels of several antipsychotic drugs, including clozapine and olanzapine.(84)

Evidence: A retrospective cohort study of 17,629 patients found a significant reduction in healthcare utilization, hospital readmissions, and deaths when patients were discharged with home healthcare vs discharged to home without home healthcare.(85)

Evidence: A retrospective cohort study based on 1151 Medicare accountable care organization (ACO) patients suggests utilizing a transitional care planning approach to prioritize discharging patients to the least restrictive next level of care, as patients discharged to skilled nursing facilities were almost 5 times more likely to be readmitted to the hospital at 30 days compared to patients discharged to the home healthcare setting.(86)

Quality standards: Standards indicate that programs should have a process for identifying barriers to patients meeting goals or adhering to the case management plan and for following up with patients on the status of referrals.(69)

Quality standards: Standards indicate that programs should have a process for identifying barriers to patients meeting goals or adhering to the case management plan and for following up with patients on the status of referrals. Programs should ensure that during care transitions the care plan is re-evaluated, modified, and communicated to appropriate care team members (eg, providers, caregivers) as needed to support the care transition.(69)

Quality standards: Standards indicate that caregiver resources should be evaluated. Programs should ensure that during care transitions the care plan is re-evaluated, modified, and communicated to appropriate care team members (eg, providers, caregivers) as needed to support the care transition.(69)

Quality standards: Standards indicate that programs should have a process for identifying barriers to patients meeting goals or adhering to the case management plan and for following up with patients on the status of referrals. During transitions, medications should be documented with the patient and designee, and discrepancies should be communicated to the appropriate provider.(69)

Outcomes: Several studies support the role of the pharmacist in medication reconciliation to identify and resolve pharmacy-related problems and improve outcomes (eg, prevention of medication-related adverse events, discontinuation of inappropriate medication).(87)

Evidence: A prospective cohort study of 2655 hospitalized patients after discharge found that 24% of hospital medication changes were not followed, 30% of changed doses were filled with the incorrect dose, 27% of new medications were not filled, and 12% of discontinued medications were filled. Risk factors for medication change problems included increased out-of-pocket costs, not having medications filled before hospitalization, and discharge to a long-term care facility.(88)

Quality standards: Standards indicate that programs should collaborate on transitions with the discharge team, reassessing the appropriateness as needed; communicate transition plan progress with the patient and designee; and communicate care plan information with the receiving setting within a designated time frame. Programs should develop and communicate an individualized case management plan with the patient, facilitate and follow up with referrals, and assess the patient's progress with the care plan. Programs should ensure that during care transitions the care plan is re-evaluated, modified, and communicated to appropriate care team members (eg, providers, caregivers) as needed to support the care transition.(69)

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Footnotes

[A] The timing of provider follow-up depends on the patient's disease severity, self-care complexity, patient/caregiver willingness and ability to provide patient care, and available social supports.(50) [A in Context Link 1]

[B] Adherence in the early stages of the disease has an opportunity to influence its chronic and progressive course.(13) Early intervention in psychosis services can improve clinical outcomes, including admission rates, symptoms, and relapse rates, for people with a first episode of psychosis.(15) [B in Context Link 1]

[C] Electroconvulsive therapy (ECT) may be useful for psychotic symptoms, particularly if the patient is not responsive to pharmacologic treatment.(89) [C in Context Link 1]

[D] Cocaine, marijuana, and amphetamines can provoke psychotic symptoms in patients with schizophrenia.(14) [D in Context Link 1]

[E] Telehealth may improve access to and coordination of mental health care. Patients with behavioral health diagnoses may use telehealth or telepsychiatry to access mental health support or care (eg, to get assistance with problem-solving techniques or ask about presenting symptoms or medication use).(90) [E in Context Link 1]

[F] Neuroleptic malignant syndrome is a rare, rapidly progressive, potentially fatal complication more common with typical antipsychotics. Onset can occur hours or years after initiation of drug therapy.(61) [F in Context Link 1, 2]

[G] Tardive dyskinesia (TD) is more common with first-generation (typical) antipsychotics. The risk of TD is higher with long-term use of antipsychotics. The offending medication should be removed with the first sign of symptoms, as symptoms may be irreversible.(62) [G in Context Link 1]

[H] Use typical antipsychotic medication when psychotic symptoms outweigh the adverse effects of medication.(63) [H in Context Link 1]

Codes

ICD-10 Diagnosis: F20.0, F20.1, F20.2, F20.3, F20.5, F20.81, F20.89, F20.9, F25.0, F25.1, F25.8, F25.9

DSM-5: F20.81, F20.9, F25.0, F25.1

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Transitions of Care 28th Edition
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Last Update: 3/14/2024 1:20:34 AM
Build Number: 28.1.20240313235330.016020